

Special Supplemental Nutrition Program for Women, Infants, and Children (WIC)

The Special Supplemental Nutrition Program for Women, Infants, and Children, popularly known as WIC, provides nutritious foods, counseling on healthy eating, breastfeeding support, and health care referrals to more than 6 million low-income pregnant, postpartum, and breastfeeding individuals, infants, and children at nutritional risk — and leads to long-term benefits.

Why Is WIC Important?

Infants and very young children can face lifelong cognitive and health consequences if they don't get adequate nourishment. WIC aims to ensure that pregnant individuals get the foods they need to deliver healthy babies and that those babies are well-nourished as they grow into young children.

Extensive research over the last few decades shows that WIC works. WIC participation contributes to healthier births, more nutritious diets, improved infant feeding practices, better health care for children, and higher academic achievement for students.

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How Is WIC Funded?

WIC is federally funded through the annual appropriations process; states are not required to contribute funds. Since 1997, Congress — on a bipartisan basis — has provided sufficient funding each year for WIC to serve all eligible applicants. The program received approximately \$7 billion in fiscal year 2024.

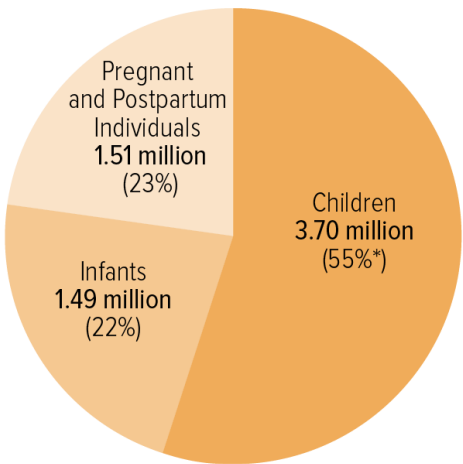
Who Is Eligible for WIC?

Pregnant, postpartum, and breastfeeding individuals, infants, and children under age 5 are eligible if they meet income guidelines and if a health care professional, such as a registered dietitian or nurse, or a trained nutrition paraprofessional has determined them to be at “nutritional risk.” Generally, applicants who meet the income requirements have a medical or dietary condition that places them at nutritional risk.

All postpartum individuals who meet the income guidelines and nutritional risk criteria are eligible for WIC benefits for up to six months after childbirth; individuals who continue to breastfeed their infants beyond six months are eligible for WIC benefits for up to a year after childbirth. Young children receive WIC benefits for up to a year, after which eligibility must be redetermined.

Applicants who receive no other relevant means-tested benefits must have gross household income at or below 185 percent of the federal poverty level ([currently](#) \$47,767 annually for a family of three) to qualify for WIC benefits. To simplify program administration, an applicant who already receives Supplemental Nutrition Assistance Program benefits (SNAP, formerly food stamps), Medicaid, or monthly Temporary Assistance for Needy Families cash assistance payments is automatically considered income-eligible for WIC. [Four in five](#) people approved for WIC benefits (80.4 percent) receive one of these other benefits.

WIC Serves More Than 6 Million Low-Income Infants, Children, and Pregnant or Postpartum Individuals



	Number of participants	Share of total participants
Children		
4 years old	0.5 million	7.1%
3 years old	0.8 million	12.2%
2 years old	0.9 million	14.1%
1 year old	1.4 million	20.8%
Infants	1.5 million	22.2%
Adults		
Pregnant	0.5 million	8.2%
Breastfeeding	0.6 million	9.1%
Non-breastfeeding postpartum	0.4 million	5.3%

*Includes children for whom no age data are available.

Source: CBPP analysis of U.S. Department of Agriculture (USDA) administrative data for fiscal year 2024. The number of children aged 1 to 4 is estimated using USDA, “WIC Participation and Program Characteristics 2022”

How Do Families Apply for WIC?

Pregnant individuals and parents of young children are often referred to WIC by their doctor or when they apply for Medicaid, SNAP, or other community services. They can also apply directly for WIC benefits at one of WIC's [10,000 local clinics](#), located in community health centers, Indian Health Services clinics, hospitals, and local health departments. While many applicants contact WIC clinics by telephone to request services, state and local agencies are increasingly making applications or forms to request WIC benefits available online.

Applicants are scheduled for certification appointments to determine eligibility and food benefits, during which they are asked to provide documentation of where they live, their identity, and their income or receipt of other qualifying benefits. They also meet with a professional or paraprofessional for a nutrition assessment to identify nutritional risks. Federal rules require applicants to be present during these appointments, though under certain circumstances local staff may exempt newborn infants, children with working parents, and individuals with health conditions that prevent them from attending in-person appointments. However, under [federal waivers](#) in effect through September 2026, some WIC agencies currently allow all applicants to conduct appointments by telephone or videoconference rather than in person. [Participants have responded very favorably to remote appointment options](#), which reduce the transportation, work schedule, and child care barriers associated with in-person appointments.

WIC agencies have also set up various methods for applicants or participants to submit documents (or photos of documents) electronically, including email, text message, participant portals, document uploading tools, and WIC mobile apps. The 2021 American Rescue Plan funded program modernization and innovation. (See “How is WIC modernizing?” below.) WIC agencies can apply lessons learned from these investments and adopt promising practices to make ongoing improvements in operations. By [continuing and expanding practices](#) to make WIC benefits and services easier for families to access and use, agencies can improve the reach and impact of the program in the coming years.

How Does WIC Provide Services?

The Department of Agriculture's (USDA) Food and Nutrition Service oversees WIC at the federal level and provides funds to state health departments, Indian Tribal Organizations, Washington, D.C., and U.S. Territories to provide WIC benefits and administer WIC programs. States, in turn, allocate or use the funds to staff local WIC clinics, which provide participants with services and electronic benefit cards to purchase nutritious foods. The services that local WIC clinics provide include individual nutrition counseling, nutrition classes, breastfeeding support, smoking cessation support, and referrals for health care and social services.

What Foods Does WIC Provide?

WIC is not meant to provide the full array of foods that a pregnant individual or a young child needs. Instead, it provides specific types of foods chosen through a rigorous, science-based process because they tend to be lacking in the diets of low-income women and young children. The program provides a limited number of foods — such as whole grain bread, baby food, infant formula, and milk — as well as separate “cash value benefits” that can be used only to buy fruits and vegetables.

In 2021, the cash value benefit amount was increased to provide participants with additional fruits and vegetables, consistent with recommendations from the National Academies of Sciences, Engineering, and Medicine (NASEM). In fiscal year 2025, children received \$26 monthly for fruits and vegetables, pregnant and postpartum participants received \$47 monthly, and breastfeeding participants received \$52 monthly. A [2024 update](#) to WIC’s food packages implemented other NASEM recommendations to increase the variety of foods in WIC food packages in alignment with the latest nutritional science and to address key nutritional needs to support healthy dietary patterns.

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How Do Participants Receive WIC Foods?

Approximately [47,000 grocery stores](#) nationwide have been approved to redeem WIC food benefits based on their prices and the variety of foods they offer. Participants select their WIC foods from the shelves and use WIC electronic benefit cards to pay at the register. The state WIC program then reimburses the store for the retail value of the WIC foods.

In 2023, WIC [completed a transition](#) from paper vouchers to electronic benefit cards to provide food benefits. The benefit cards simplify WIC transactions in the checkout line, eliminate the stigma of paying with paper vouchers, and allow for stronger program management and oversight. Electronic benefits can be loaded remotely onto “online” cards but not “offline” cards. Because online cards facilitate remote services and updates to participants’ food packages, states that have been using offline cards are transitioning to online cards.

The transition to electronic benefit cards facilitates other modernization efforts to make shopping for WIC foods easier by employing options available to all consumers. When offered the opportunity to use self-checkout, curbside pick-up, drive-through windows, and special WIC food sections in stores, [participants use these options and report favorable experiences with them](#).

Both participants and grocers are [eager](#) for additional ways to make WIC shopping more equitable and convenient. USDA has initiated efforts to shift WIC toward online shopping platforms, including grant-funded

projects to test and evaluate online ordering and the creation of a congressionally required [Task Force on Supplemental Foods Delivery](#). The Task Force — composed of a broad range of stakeholders, including WIC providers, retail grocers, electronic benefit transfer processors, and food manufacturers — recommended measures to ensure that WIC participants can access the same variety of online shopping options as other shoppers without stigma, added difficulty, or personal cost. [Pilot projects](#) exploring ways to make online shopping available to WIC participants are underway. Efforts to improve and modernize the WIC shopping experience are crucial for enabling participants to fully use their food benefits and maximize the health benefits from participating in WIC.

How Does WIC Support Infant Feeding?

WIC promotes breastfeeding as the optimal feeding choice for infants, unless it is inadvisable for medical reasons, so participants get the health, nutrition, and emotional benefits that research shows breastfeeding provides. WIC supports breastfeeding through education and peer counseling and by providing exclusively breastfeeding individuals with an enhanced food package, longer eligibility, and breast pumps. WIC provides infant formula for infants who are not breastfed.

How Does WIC Obtain Infant Formula?

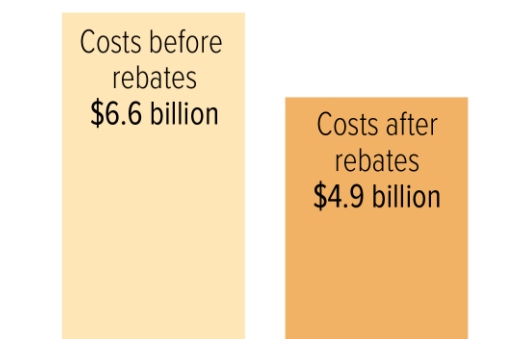
WIC purchases of infant formula account for more than half of U.S. formula consumption because WIC serves approximately 40 percent of all infants in the country. WIC uses a [competitive bidding process](#) in which infant formula manufacturers offer discounts, in the form of rebates, to state WIC programs in order to be selected as the sole formula provider to WIC participants in the state, except when families need specialized formula for medical reasons. This process, adopted through bipartisan legislation in 1989, saves the federal government roughly \$1.5 billion annually.

As a result of these savings, WIC's cost to the government is much lower than the full retail value of WIC benefits for program participants. These savings have played an important role in allowing WIC to serve all eligible applicants with the limited funding the program receives through the annual appropriations process.

To help families cope with the 2022 formula shortage precipitated by a safety recall, the federal government temporarily expanded state flexibility over formula purchases by WIC participants. WIC contracts with infant formula companies are now [required](#) to include

Infant Formula Rebates Reduce Federal WIC Food Costs Considerably

WIC food costs, fiscal year 2024



Source: U.S. Department of Agriculture administrative data for fiscal year 2024

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provisions to mitigate the impact of a recall on WIC participants, such as by letting them select another size or brand of formula.

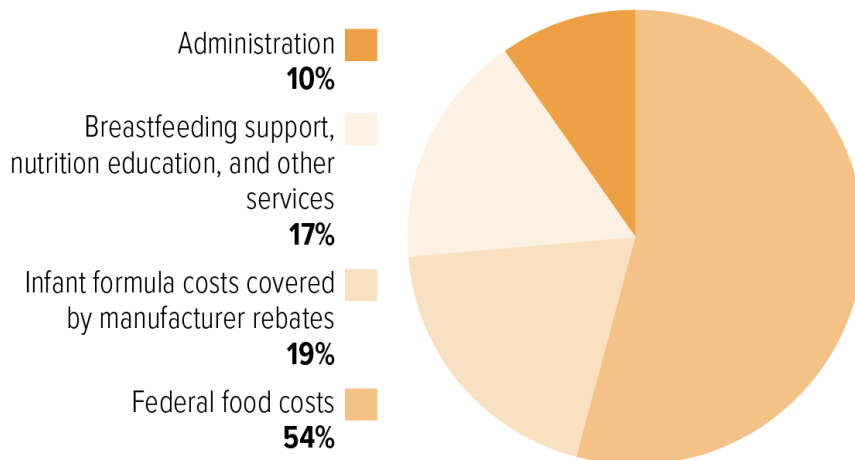
How Much Do Households Receive in Food Benefits?

WIC provided an average value of \$81.51 in food per participant per month in fiscal year 2024. The average monthly cost to the [federal government](#), however, was much lower — \$61.10 per participant — due to the infant formula discounts discussed above. Because many infants receive infant formula, which is the most expensive item WIC provides, the average monthly value of the WIC benefit for infants is higher than the average monthly value for pregnant and postpartum individuals and children.

How Are WIC Funds Spent?

The competitive bidding process used to procure infant formula, described above, leverages market forces to reduce WIC food costs. WIC funds not spent on food support the staff and operating costs of providing nutrition education, breastfeeding support, and other services — a key part of the program’s success. WIC’s administrative costs represented 10 percent of total program costs in fiscal year 2023, consistent with their share in recent decades. By law, WIC funding per participant for nutrition services and administration combined may rise no faster than inflation.

Only 10 Percent of WIC Costs Go Toward Administration



Source: CBPP analysis of U.S. Department of Agriculture administrative data for fiscal year 2023

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How Effective Is WIC?

Extensive research shows that WIC contributes to [positive developmental and health outcomes](#) for low-income pregnant, postpartum, and breastfeeding individuals and young children. In particular, WIC participation is associated with:

- **Healthier births.** Prenatal WIC participation helps pregnant individuals give birth to healthier infants and helps lower infant mortality rates.
- **More nutritious diets.** WIC has helped reduce the prevalence of anemia, and strong evidence suggests that WIC participation increases infants' and children's intakes of some essential vitamins and minerals and improves infant feeding practices. Following the introduction of improved WIC food packages more closely aligned to current dietary guidance and increases to the cash value benefit for purchases of fruits and vegetables, WIC participants buy and eat more fruits, vegetables, whole grains, and low-fat dairy products.
- **Stronger connections to preventive health care.** Children in families with low incomes participating in WIC are just as likely to be immunized as those from higher income families. And they are more likely to receive preventive medical care than other children in families with low incomes.
- **Improved educational prospects.** Children whose parent participated in WIC while pregnant scored higher on assessments of mental development at age 2 than similar children whose parent did not participate. Later, they also performed better on reading assessments in school.
- **Greater equity.** Evidence suggests that participation in WIC reduces racial and ethnic disparities in birth outcomes and breastfeeding.

Despite these proven benefits, WIC reaches only slightly more than [half](#) of eligible people (53.5 percent in 2022, the most recent year for which USDA has published estimates), and fewer than half of eligible 2- to 4-year-old children and pregnant adults. Take-up [varies widely across states](#), ranging from 37 to 73 percent of eligible individuals enrolled in WIC.

Pregnancy-related complications and deaths, among both parents and infants, are persistent and strikingly high compared to other developed countries, especially among Black and Latino people. Because WIC participation is associated with [reduced risk](#) of premature birth, low birth weight, and infant mortality, increasing the share of eligible families who participate in WIC — including pregnant people of color and their infants, who face greater health risks — can be part of a broader strategy to improve maternal and child health outcomes overall and mitigate racial and ethnic disparities in such outcomes. Expanding WIC participation could also play an important role in mitigating food hardship, especially for Black and Latino children, who face food insecurity at much [higher rates](#) than white children.

How Is WIC modernizing?

In recent years, state and local WIC agencies have implemented new digital tools and streamlined processes to make the program more efficient and easier to use. For example, the transition from paper vouchers to electronic food benefits was a major improvement for both the families receiving WIC and the grocery stores

Policy Basics – Special Supplemental Nutrition Program for Women, Infants, and Children

serving WIC customers. Flexibilities implemented during the pandemic and an infusion of funding for WIC modernization in 2021 supported innovation, expanded the use of technology, and encouraged adoption of more efficient ways for eligible families to enroll in WIC and utilize the program's services to improve their health.

In most states, families can now start the application process for WIC online, share enrollment documents and household information electronically, communicate with WIC via text messaging, and participate in telehealth appointments for nutrition and breastfeeding counseling. These flexibilities are especially important for families juggling work schedules and child care arrangements or facing transportation challenges.

Ongoing efforts to develop and replicate innovative modernization and streamlining approaches will help ensure WIC continues to improve participants' health through cost-effective services and will help WIC reach more eligible families.

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